

HEMATOLOGY PROGRESS NOTE

New patient consultation

Patient Priya Raman · 41F · DOB 1984-11-03
Patient ID PT-4471118
Primary diagnosis D50.9 · Iron deficiency anemia, unspecified

SUBJECTIVE

The patient is referred for evaluation of anaemia found on routine screening. She reports six months of progressive fatigue, exertional dyspnoea climbing two flights of stairs, and cold intolerance. She describes heavy menstrual bleeding for the past two years, lasting seven days with clot passage. She has no melena, haematochezia, or haematemesis. She tried ferrous sulfate 325 mg twice daily for eleven weeks beginning 2026-05-04 and stopped it because of persistent nausea and constipation despite dose reduction and food timing changes. Haemoglobin did not improve on oral therapy.

OBJECTIVE

Pale conjunctivae. No scleral icterus. Cardiovascular examination shows a soft systolic flow murmur at the left sternal border, no gallop. Lungs clear. Abdomen soft, non-tender, no organomegaly. No lymphadenopathy. Koilonychia of both thumbnails. Weight 61 kg.

ASSESSMENT

Iron deficiency anaemia, D50.9, haemoglobin 9.1 g/dL with ferritin 8 ng/mL and transferrin saturation 9 percent, most consistent with chronic menstrual blood loss. Oral iron failed on tolerability grounds after an adequate eleven week trial. Total iron deficit calculated at approximately 1,450 mg by the Ganzoni method at a target haemoglobin of 14 g/dL. Intravenous replacement is indicated.

PLAN

Ferexa (ferric carbolate) 750 mg intravenously, two doses separated by at least seven days, at Cascade Infusion Center. Enrolled through ArvelisConnect for benefits verification. Repeat CBC and iron studies four weeks after the second dose. Referred to gynaecology for management of menorrhagia. Advised to stop oral iron.

ELECTRONICALLY SIGNED BY	DATE / TIME
Sofia Marchetti, MD	2026-08-25 14:02

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